

PIRIMAT INJECTION

DESCRIPTION: Clear, colourless liquid.

COMPOSITION:

PIRIMAT INJECTION (1ml amp): Each ml contains Chlorpheniramine Maleate BP 10 mg in 1 ml ampoule.

PIRIMAT INJECTION (10ml vial): Each ml contains Chlorpheniramine Maleate BP 10 mg in 10ml vial (Preservative: Phenol 0.5%)

PHARMACODYNAMICS: H₁-Receptor Antagonist. Antihistamine diminish or abolish the main actions of histamine in the body by competitive, reversible blockade of histamine receptor sites on tissues; they do not inactivate histamine or prevent its synthesis or release. Histamine receptors are classified into two main groups. H₁-Receptors are responsible for vasodilation, increased capillary permeability, flare and itch reactions in the skin, and to some extent for contraction of smooth muscle in the bronchi and gastro-intestinal tract. H₂-Receptors are responsible for gastric secretions and contribute to vasodilation. H₁-Antagonist also possess antimuscarinic, adrenaline-enhancing and/ or antagonising, serotonin-antagonising and local anaesthetic effects. H₁-Antagonist is used for the palliative treatment of allergic reactions.

PHARMACOKINETICS: Chlorpheniramine is well absorbed after oral administration. Protein binding of chlorpheniramine is 70%. The half-life elimination is about 14 to 25 hours. Onset of action for most antihistamines is about 15 to 60 minutes. Time to reach peak concentration for oral chlorpheniramine is 2 to 6 hours, and to reach peak effect takes about 6 hours. Chlorpheniramine a derivative of propylamine has a duration of action from 4 to 8 hours. Most antihistamines are excreted as metabolites within 24 hours.

INDICATION: For all conditions requiring antihistamines therapy such as hay fever, vasomotor rhinitis, urticaria, angioneurotic oedema, food allergies, drug and serum reactions, atopic and contact dermatitis, allergic eczema, pruritus ani et vulvae, insect bites, and some cases of asthma.

RECOMMENDED DOSAGE: The injection may be subcutaneous, intramuscular or intravenous.

When a rapid effect is desired, as in anaphylactic reactions, the intravenous route is recommended in addition to emergency therapy with adrenaline (epinephrine), corticosteroids, oxygen and supportive therapy as required. In this case chlorpheniramine injection should be injected slowly over a period of one minute, using the smallest adequate syringe. Any drowsiness, giddiness or hypotension which may follow is usually transitory.

Usual dose for adults: 10-20mg, but not more than 40mg should be given per 24 hours.

Usual dose for pediatric: Subcutaneous, 87.5mcg per kg of body weight for every six hours as needed.

Chlorpheniramine injection may be helpful in the prevention of delayed reactions to penicillin and other drugs when given separately by intramuscular injection immediately prior to administration of the other drug. The usual dose is 10mg.

Chlorpheniramine injection cannot, however, be relied on to prevent anaphylactic reactions in patients known to be allergic to a particular drug.

ROUTE OF ADMINISTRATION: For i.m., s.c., or slow i.v. inj.

CONTRAINDICATIONS: Contraindicated in patients who are hypersensitive to antihistamines and those that have had MAOI therapy within the previous 14 days. Chlorpheniramine should not be used in premature or full-formed neonates

WARNINGS & PRECAUTIONS:

Warnings: Alcohol potentiates the action of antihistamines and drowsiness may occur in patients not otherwise subject to it if alcohol is taken.

Patients should be warned that antihistamines might enhance the sedative effect of central nervous system depressants including alcohol, barbiturates, hypnotics, narcotic analgesics, sedatives and tranquillizers. Since antihistamines have some anticholinergic properties they should be used with care in conditions liable to be exacerbated or otherwise adversely affected by atropine such as glaucoma and prostatic hypertrophy. Also the effects of anticholinergic drugs such as atropine and tricyclic antidepressants, may be enhanced by concomitant administration of those antihistamines possessing similar activity. It has been suggested that some antihistamines could mask warning symptoms of damage caused by ototoxic drugs, such as the aminoglycoside antibiotics. Some antihistamines have been reported to affect the metabolism of drugs in the liver.

Caution: Antihistamines may cause drowsiness and dulling of mental alertness, patients undergoing treatment with these drugs should not take charge of vehicles, other means of transport, or machinery where loss of attention may lead to accidents. Patients should abstain from alcohol.

Safety and efficacy of chlorpheniramine maleate injection in children below the age of 12 years old have not been established.

INTERACTION WITH OTHER MEDICAMENTS:

Incompatibilities: The injection is incompatible with iodipamide meglumine, calcium chloride, kanamycin sulphate, noradrenaline acid tartrate, pentobarbitone sodium. There was a loss of clarity when intravenous solutions of chlorpheniramine maleate were mixed with those of calcium chloride noradrenaline acid tartrate or phenobarbitone sodium. MAOI therapy intensifies the anticholinergic effects of chlorpheniramine. Chlorpheniramine inhibits phenytoin metabolism and can lead to phenytoin toxicity.

USE IN PREGNANCY & LACTATION:

Use in Pregnancy: Chlorpheniramine maleate injection should be used during the first two trimesters only when clearly needed. Because of the risk of severe reaction (eg. seizures) to antihistamines in neonates, chlorpheniramine should not be used during the third trimester.

Breast Feeding: First generation antihistamines may inhibit lactation because of their anticholinergic actions. Small amount of antihistamines are distributed into breast milk; used is not recommended in nursing mothers because of the risk of adverse effects, such as unusual excitement or irritability, in infants.

SIDE EFFECTS: Adverse effects of Antihistamines (H₁-Receptor Antagonist). Side effects with antihistamines vary in incidence and severity with each patient as much as with each drug, though some of the drug gives rise to more side effects than others. The most common effect, cause to some extent by all its sedation, varying from slight drowsiness to deep sleep, and including inability to concentrate, lassitude, dizziness, hypotension, muscular weakness, and incoordination. Patient should be warned that if drowsiness and dizziness occur, they should not drive a car or operate machinery. Sedative effects, when they occur, may diminish after a few days. Other side effects include gastro-intestinal disturbances such as nausea, vomiting, diarrhoea or constipation, anorexia, increased appetite and epigastric pain. Antihistamines may also produce headache, blurred vision, tinnitus, elation or depression, irritability, nightmares, anorexia, difficulty in micturition, dryness of the mouth, tightness of the chest and tingling, heaviness, and weakness of the hands. Paradoxical CNS stimulation may occur especially in children, with insomnia, nervousness, tachycardia, tremors, and convulsions. Antihistamines may precipitate epileptiform seizures in patients with focal lesions of the cerebral cortex. Symptoms of stimulation may also arise in some adults, especially after taking phenindamine, but occasionally with other antihistamines too. They include insomnia, nervousness, tachycardia, tremors, muscle twitching, and convulsions. Large doses of antihistamines may precipitate fits in epileptics. There have been reports suggesting a possibility of human foetal abnormalities resulting from the use of some antihistamines, especially the piperazine derivatives, but a causal relationship has largely been rejected.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Overdose may be fatal especially in infants and children in whom the main system are CNS stimulations and antimuscarinic effects, including ataxia, excitement, hallucinations, muscle tremor, convulsions, dilated pupils, dry mouth, flushed face, and hyperpyrexia. Deepening coma, cardiorespiratory collapse, and death may occur within 18 hours. In adults, the usual symptoms are of CNS depression with drowsiness, coma, and convulsions. Hypotension may also occur, elderly patients are more susceptible to the CNS depressant and hypotensive effects even at therapeutic doses.

Treatment: Treatment includes gastric lavage. Emetic may be tried (using syrup of Ipecacuanha) if the patient is alert and there are no symptoms of toxicity, but may be ineffective due to the antiemetic activity of antihistamines. Following these measures, activated charcoal and cathartics may be administered to minimize absorption. Convulsions may be controlled with diazepam. Central stimulants may increase the risk of convulsions, and their avoidance is advisable. Other treatment is supportive and symptomatic and may include artificial respiration, external cooling for hyperpyrexia, and intravenous fluids. Vasopressors such as Noradrenaline or phenylephrine may be used, but adrenaline must not be given as it can paradoxically lower the blood pressure further.

Forced diuresis is of little value since many antihistamines are rapidly metabolized. The benefits of peritoneal dialysis or haemodialysis appear to be limited.

STORAGE CONDITIONS:

PIRIMAT INJECTION (1ml amp): Store below 30°C. Protect from light and freezing. KEEP OUT OF REACH OF CHILDREN. JAUHKAN DARIPADA KANAK-KANAK.

PIRIMAT INJECTION (10ml vial): Store below 30°C. Protected from light and freezing. KEEP OUT OF REACH OF CHILDREN. JAUHKAN DARIPADA KANAK-KANAK.

Stability after opening for PIRIMAT INJECTION (10 ml vial): Please store in refrigerator for not more than 4 weeks. Do not freeze.

PACK SIZE: Ampoules: 10 ampoules x 1 ml,

Vials: 10 x 10ml vials, 25 x 10ml vials

SHELF LIFE: Please refer to outer package.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.